



NAKKY
ACADEMY

Phone: +27 67 177 8439
Address: 7 Island View Bluff
Fynland, Durban

AU PAIR & ELDERLY CARE GIVING

A Comprehensive Course

2023-2024



MODULE TWO: TAKING CARE OF THE ELDERLY

After completing this module you'll be able to;

- **Understand Frail Care**
- **Communicate with a Person with Dementia**
- **Handle Troubling Behavior**
- **Manage Wandering**
- **Deal with Incontinence**
- **Control Agitation**
- **Handle Repetitive Speech or Actions (Perseveration)**
- **Know how to deal with Paranoia**
- **Manage Sleeplessness/Sundowning**



What is frailty?

The clinical condition of ‘**frailty**’ is one of the most-challenging consequences of population ageing. **Frailty** develops as a consequence of **age-related decline** in multiple body systems, which results in **vulnerability to sudden health status** changes triggered by minor stress or events such as an infection or a fall at home. Between a quarter and half of people older than 85 are estimated to be frail, with overall prevalence in people aged 75 and over approximately 9%.

People with frailty have a substantially **increased risk of falls, disability, long-term care and death**. We also know that frailty is a graded abnormal health state which ranges from the majority who are **mildly frail** and need **supported self-management**, through those who are **moderately frail** and would benefit from **interventions** such as case **finding/case management**, to those who have **advanced frailty** where **anticipatory care** planning and **end-of-life care** may be appropriate interventions. So frail people should not be perceived as a problem to the system but, rather, caregivers should support people with living with frailty to maintain their own health for as long as possible.



Why Frail Care?

Frail care is needed when you can no longer take care of yourself, physically or mentally. This implies that you need help with normal daily tasks such as eating, personal hygiene and moving around, or that your mental condition is such that you need supervision.

Frail care is associated with growing older. The reality can therefore not be ignored. Being prepared can make a great difference to how we deal with the situation. People often think that if they have made provision to move into a retirement community, they have made provision for frail care as well. Unfortunately, this depends on the specific retirement community. More often than not there are costs involved, for example needing help in your home or having to move into the frail care center of your retirement community. Medical aid cover was not designed to cover on-going frail care. It may cover a short-term period of urgent medical care (for instance recovering from a stroke, a heart attack or a hip fracture) but on-going frail care support is not considered part of medical aid cover. Some specific medical aid schemes may cover some frail care costs, but the cover is limited and the costs are high.

Frail care is provided by Caregivers or Nursing staff, depending on what may be required.

Caregivers/Nursing staff offers the following forms of care;

- **Acute care**

Is usually required for a short period of time when a patient needs urgent medical care.

- **Primary care**

Is on-going medical care for patients with chronic diseases for instance diabetes, high blood pressure and asthma.

- **Palliative care**

Focuses on reducing the severity of the impact of the symptoms of a disease, not on curing it. Diseases such as cancer, renal disease and chronic heart failure may need palliative care which mainly attempts to relieve suffering by pain management. Palliative care also provides some quality of life as well as support to the individual and his/her family.

Types of Facilities Which Offer Frail Care

- **Acute care facility**

Care takes place in a licensed and registered facility with a practice code number, for example hospitals and clinics. A hospital has operating theatres and intensive and/or high care units. A day clinic, which has an operating theatre but is not equipped for overnight care, is also regarded as an acute care facility, as is a mental health hospital. Cover for care in these facilities would normally form part of a medical aid benefit. Other levels of care are more problematic and are either not covered by the medical aid at all or not covered sufficiently

- **Sub-acute care facility**

This facility has registered nursing staff, but lacks facilities such as operating theatres. This type of facility will provide high-quality nursing care, but not the specialized (high-tech) care associated with a hospital. A sub-acute care facility is for a patient whose condition has stabilized; someone who no longer needs hospital level care, but still requires professional care. Some facilities focus on psychiatric conditions and others on physical rehabilitation. Sometimes the term step-down facility is used to describe a sub-acute facility. Depending on your medical aid scheme and especially on your condition, some of the care provided in a sub-acute facility may be covered.

- **Assisted living**

Also known as supported living. In most cases, a person will live in his or her own unit in a retirement community, but will have meals supplied, as well as being supported by cleaning services, a care giver assisting with personal hygiene and the administration of medicine. A nurse may also be on 24-hour call.

- **Home-based care**

Many villages offer home-based care, which delays the need for frail care, because frail care, with staff on duty day and night, is expensive. Once you need 24-hour care, however, frail care is cheaper than home-based care. Some villages prefer to promote home-based care and the concept of “ageing in place” – or ageing in your own home and community. Keeping people in their own communities means that only the services that are needed, are purchased from a care provider. For example, if a healthy spouse is not strong enough to provide for all his or her ill partner’s needs, the couple can make use of

home-based services to help with something like bathing, and the healthy spouse can continue to do whatever he or she is still capable of doing, such as shopping and cooking.

IMPLICATIONS OF FRAILTY ON THE ELDERLY

1. Impact on the individual

Frailty affects all aspects of a person's life: Physical, mental, social and economic. For every person, this is a unique journey where some become weaker than others, and different life functions are impacted. All elderly become weaker due to the normal ageing process, but inadequacies mainly on physical, cognitive, social and psychological levels makes it more difficult for some to function independently, and results in increasing levels of dependency. Disability in frail elderly is common due to age-related physical and mental impairment; visual and auditory decline; decrease in mobility, gait, muscle and bone strength; poorer sensory perception and responses to environmental stimuli, as well as any medical and neurological conditions that cause physical disability.

2. Physiological factors

As frail elderly become weaker, they have reduced physiological reserves in all the systems of the brain; the endocrine organs; immune system; skeletal muscles; and lower cardiovascular, respiratory and renal functions. Physical deterioration is mostly caused by a progressive decline in skeletal muscle mass. The physical inactivity is the main reason for physical disability, which leads to a greater need for medical and mental health care, as well as social services. Frailty requires assistance that reaches beyond mere physical factors, and includes aid within a person's psychological dimensions, cognitive performance, mood conditions, social care support, and attention to the living environment.

This deterioration leads to the need for physical care. Physical care is referred to as personal or intimate care, with assistance in the basic ADL (Activities of Daily Living). As every frail elder is unique, with individual needs, the level of ADL dependency such as bathing; dressing; toilet

and incontinence management; transferring from bed to chair and back to bed and feeding / eating will differ from person to person. For religious, moral or personal reasons, providing physical care is a sensitive matter in all cultures. Specific care needs should be identified and serviced by multiple providers to ensure that the needs are met as fully as possible. Care needs at home will differ, depending on individual and gender preferences, such as who should provide assistance.

3. Care related to cognitive impairment

Cognitive impairment in the frail elderly is associated with structural and physiological changes in the brain. Dementia like Alzheimer's disease has been identified as a key component of the stress response of frail elderly due to the decline in their cognitive functions. Frailty combined with dementia would indicate that the frail elderly is a high-risk group with adverse outcomes, such as wandering and other behavioral problems.

Behavioral problems associated with cognitive impairment become more common the older the frail become, including wandering around at night; uncontrolled screaming or crying; confusion and disorientation; asking the same questions repeatedly, and aggressiveness towards caregivers. Aggressive behavior is one of the principal problems of dementia. Unacceptable or inappropriate behavior such as masturbation, increased libido, playing with faeces or urinating in public places can be challenging for caregivers. Studies have found that sexually inappropriate behavior is part of all stages of dementia and is common in communities of frail elderly with this condition.

4. Safety and security

Home safety and security has a wide range of implications for the frail elderly, including prevention of accidents in the house, and accidents caused by internal or external factors such as crime. The frail elderly mostly prefers to stay at home, and often prefer to die in their own homes. An effective system of caring for this growing frail elder population needs an economic solution, however, and the only really affordable option is that frail elderly do remain in their homes for long as possible, as advised. Nonetheless, it is important to consider whether this is the safest option for this vulnerable population. It is important to maintain and improve frail elderly's safety and independence at home.

5. **Medication control**

Medication compliance is often not optimal among all people on treatment, but especially the use of chronic medication. However, taking chronic medication is a daily reality for most of the frail elderly. Frail elderly also presenting with dementia often demonstrate poorer medication adherence due to their cognitive limitation. Illiteracy or impaired eyesight is another obvious problem, resulting in incorrect dosage; forgetting to take medication, or duplicating dosages more than once.

6. **Abuse**

Elderly persons at risk of abuse are often female; physically and / or cognitively impaired; experience behavioral problems that cause distress such as wandering; urine and stool incontinence; night-shouting or confusion. Another category includes any elder dependent on assistance; or those who are dependent yet maintain an autocratic role towards their adult children. The risk for abuse can be physical, sexual (rape), verbal, and / or financial.

